

WORKPLACE COUNSELLING SERVICES

EDUCATIONAL INSTITUTIONS COUNSELLING

CRISIS COUNSELLING

MANAGEMENT OF COUNSELLING SERVICES

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INTRODUCTION

Mental Health is an integral part of human functions. Work place counselling provides enormous benefits to both the employee and the employer. The employee is able to address psychological challenges that they have hence improves on self-awareness, coping strategies and developing strategies for their problems. This in turn makes them productive employees. Hence high income for the company. Likewise, in schools, fully functional students and staff will mean less problems to their families and the school management hence better performance in academic and cocurricular activities. Crisis counselling is quite vital especially with the current economic, social and all-round emergencies that destabilize the society. A counsellor would provide crisis counselling at work place, education institutions or even in private practice. For crisis counsellor the counsellor needs extra training on crisis and trauma management for them to be effective.

. DEFINITION OF TERMS:

Counselling: It is a professional Psychotherapeutic relationship between a professional counsellor and a client. The client presents psychological problem hindering his day to interactions and hence hampering his productivity. In this course, I will use the terms Psychotherapy, Psychological services and counselling interchangeably. They all constitute mental wellbeing. (Palmer2015, & (Feltham, 2015),

Counselling services: This may include all the interventions geared at the psychological wellbeing of the clients. Examples of services include Guidance, Psychoeducation, team building, Group therapy, Short courses training and capacity Building. (Wango,2015)

Work place counselling: Counselling provided at the work place for a specific organization. It is normally factored in as part of Employee assisted Programmes (EAP) (Adeniji, A. et al 2013)

Counselling in the learning institution: Counselling provided in schools, colleges and Universities. Mainly it targets students however even the staff can also benefit from the services (Kirti, 2016)

Crisis Counselling: This is counselling provided to individuals who have encountered emergencies that are either stressful or traumatizing. The aim of crisis counselling is to calm down the client so as to focus on solving the crisis. (ACA n.d)

Management of counselling services: This is administration or overseeing of the counselling services. (Adeniji, A. et al 2013)

COUNSELLING SERVICES OFFERED

Counselling services offered at work place are, Induction, Team building, Career development training, Psycho-education, Mentorship Coaching Stress and burn out management. In addition to these Guidance is offered at schools. There is normally a programme when the group sessions are carried out. The program could either be guided by need analysis or is part of the school program as a preventive measure, for example schools can have a programme of drug and abuse awareness training to curb incidences of such in the school. It could also be a means to educate the teenagers on the dangers of drugs so as those who have started the vice can stop. At work place too team

building and psychoeducation could also be used as deterrent for prospective misbehavior or corrective measure for an existing malfunction.

These services are done in a less structured confidential way. Mostly it is done in groups either indoor or outdoor activities. However, there is need to be offered by qualified counsellors. Psychoeducation and guidance for example is more than teaching and talking but there is a need for clear understanding of such psychological constructs like personality types, motivation, group dynamics and general Human behavior functioning. All these are entrenched in the curriculum for training counsellors. Therefore, institutions and organizations need to factor in the concept of professionalism as they engage individuals to carry out these tasks.

The other form of counselling services offered is individual counselling. This is done in a more structured and confidential environment. Clients seek this form of therapy to alleviate the current suffering that interferes with normal functioning thus limiting them from achieving their set goals as individuals at work place, school or with family relationships (APA,2020, Kirti2016)). It is recommended to be done in a well-organized counselling room. The room should be well ventilated and having comfortable seats. It should be noted that a lot of guidance is done at school since the children in Primary and high school are underage so they may not be able to make solid decisions in their lives. However, the guidance should also be offered by a professional counselor for it to be effective. In the following part of this essay I will focus on the individual counselling process.

STAGES OF THE COUNSELLING PROCESS

Stage 1: Relation building /Initial Disclosure

Stage 2: Problem assessment/In depth Exploration

Stage: Setting goal/Commitment to Action

Stage 4: Involves invention/Counselling

Stage 5: Evaluations, follow up, termination of referrals

Stage 1: Relation Building and Initial Disclosure

At this stage relation building is the focus. The counsellor establishes a rapport with the client. The rapport is based on trust, respect and care. The kind of relationship established with the client at this stage determines the quality of therapy ahead. At this stage all the basic counselling skills are used. Relationship building can be built in the following ways: being warm and inviting, addressing the client by name, pick up a social communication to help them reduce anxiety, allow them to talk about the reason for their coming, indicate your interest to listen to them. At this stage the client can talk about all their experiences from childhood to the present or they can just keep quiet the counselor's tact will either encourage the client to talk or to keep quiet. The counsellor should not hurry up the client. The client should be let to move at their pace. So, the counsellor can only encourage and motivate the client to carry on by use of minimal prompts. Let them move in a natural pace. (Mometrix (2020))

An example:

Counsellor: Hello Mercy, welcome and have a seat.

Mercy (seats); thanks.

Counsellor: How may I help you?

Mercy: I have a problem with my family

Counsellor: mmh..., What is it with your family?

Mercy (Continues as the counsellor Listens)

At work place the client may have been referred by the management for counselling. so the counsellor needs to listen to the client first without interrupting. In an education getting the clients may be referred by the head of institution or the guidance and counsellor teacher may have picked the client after noticing a misnomer with them. For example, habitual lateness to school or general indiscipline. So, in such a situation when the client goes for therapy the counsellor needs to listen attentively to build a good warm relationship. Notice that in such circumstances the client may not be very willing to go for therapy. Thus, the counsellor's relationship building is of essence so that the client may open up. With crisis counselling, at times this stage is very vital because the client may even storm in the office shaken and traumatized. So, the counsellor needs to calm them down. According to Bradley University Blog on counselling (n.d) relationship, the therapist needs to build trust with the client. The initial session determines what happens in the follow up sessions. No matter the model or therapeutic technique the counsellor uses the therapeutic potential will be severely limited if the professional does not build a therapeutic alliance with the client at the initial stage (American Counselling association, 2012)

Stage 2: In-depth Exploration/Assessment

At this stage the counselor majors on problem assessment. According to Mometrix (2020) At this stage the counsellor does everything to get the information regarding the client's challenges. In this way the counsellor gathers all information relevant to the client's presenting problem. It should not feel like interrogation but the client should feel like somebody has a desire to understand their

problem and is willing to assist them. Hence according to Psychologist Martin Seligman assessment is vital because:

- i. It assists the therapist to make proper diagnosis
- ii. To aid in developing a treatment plan
- iii. Proper clarification and setting of goals
- iv. To help countercheck whether the goals of therapy have been met
- v. Proper assessment helps the counsellor to better facilitate options and alternatives in the cases presented by the client

Areas of Assessment

Biodata. These are details like name, address, gender, marital status, age occupation, family of origin among others. Apart from enabling contacting in future it also gives the counsellor an insight regarding the kind of the family background the client is from. This forms a basis of some of the presenting problem.

Presenting problem

An investigation of how the presenting problem affects the client is done. The behavioral, thoughts, and feelings effects. The length of the problem, patterns of events and does it keep on recurring. Adam O. Horvath (2005) For example, a client was referred to me from a particular company. This lady's presenting problem was that she could not respond to external stimuli. Behaviors hitherto obvious like speaking, writing, going to work, bathing going to the toilet were a problem to her. She could hardly coordinate her walking. On enquiring further about physical cause like injury, there was no any. So, on deeper exploration on when the presenting problem began, I traced it on

violence by the husband and partly loss and grief of her father. Apparently when violated by the husband, the father was her solace. He had frequently asked her to quit the abusive marriage. However, she held on. So, my hypothesis was that she was also overwhelmed by the loss and grief of the father. As at the time I was seeing her she had been rescued by a good Samaritan.

Client's lifestyle

How does the client carry on with life in terms religion, Occupation social interactions and the personality type? Any special characteristics?

Family History

This is both family of origin and the immediate nuclear family (In case the client is married.) Number of siblings, whether parents are alive, any sickness in the family, how was the upbringings, were there any issues of concern like abuse or addictions? Family roles when growing up,

Personal History

This comprises of educational history, academic performance, hobbies at school, relationship experiences during school. Jobs at hand and occupational wishes? Any personal goal in life? What motivates them, what demotivates them?

Client appearance during the session

How is their Physical appearance, kept or unkept, dressing, gestures, facial expression, energy levels, their motivation, relationship with the counselor: Are they warm, cold, passive, willing, coherence among others.

Use of Assessment tools

At times a client may present cases that will need further assessment. For example, Post Traumatic stress Disorder (PTSD) Depression, stress, personality type/Disorder. These tools will reveal accurately where the client is suffering from. The counselor, however should make sure that the tools are valid and reliable. A valid tool measures what it is supposed to measure and it is reliable if it can consistently yield the same results. There are already standardized tests like Becks Depression scale, PTSD screening tool, Big five Personality tests, CAGE for alcohol but can also be used with other addictions. There is also a standardized Diagnostic Statistical Manual of Mental Disorders (DSM 5) For example in the case above I used DSM 5 to assess her for PTSD. I did this after exploring in depth about the interventions which had been made. According to the care giver, the client had been taken to various hospitals and several tests had been conducted and no direct

medical conditions or head injuries had been detected. So, she passed for trauma assessment. The DSM 5 trauma assessment indicators are Reexperiencing, Avoidance, hyperarousal/ Hypervigilance, and negative thoughts. her case was Acute PTSD because her brain had literally shut down to avoid the traumatic thoughts. So, I referred her to a psychiatrist for treatment to enable her brain respond. We agreed with the caregiver that after she was treated, we could meet for counselling sessions to explore how the Post Traumatic Stress Disorder came about.

Findings and Conclusions

From the findings the counsellor can identify any links between the family history, personal history, the presenting problems and the assessment tool results. From there both can formulate goals of counselling and the treatment plan. On the same breath, the counsellor can ascertain

whether he needs to refer the client or can carry on with the session. In case the counsellor finds other symptoms, emergence reassessment can be done. In actual sense, assessment permeates the whole therapeutic process. Kelly & Oakes (2019) At work place and educational institutions sometimes the therapist may recommend group sessions, where the problem seems to occur among different people in the institutions. For example, burnout and stress, academic stress, chronic absenteeism, incidences of crisis in the institutions, the counsellor can recommend psychoeducation or team building groups.

Stage 3: Treatment Plan

Once the counsellor has fully grasped the client's concerns and issues, he goes ahead to write a treatment plan. It comprises of Issues: Reflecting all domains: Physical, Social, Psychological, Spiritual status and attainable goals. The client should participate in developing the plan. For proper results, the counselling relationship needs to have focus. The client focuses on problem alleviation and free from challenges. The counsellor on the other hand must focus on the problem, client's needs, the process and the goal of therapy. When setting the goals, the counsellor should ensure that they are related to the concerns of the client, they should be laid in unambiguous and quantifiable, relevant terms, within the counsellors' knowledge (If not refer the client) and focused on positive growth.

The therapist facilitates the client to make proper goals regarding the issue for example the behaviors, thoughts and feelings to rectify or adopt Tyrrel (2019). For example, at work place counselling, the client may desire to stop the mentality that she is hated by her colleagues hence absconding duty. The client may want to stop being late to work. Another may want to address her

family problems that are getting into her way of achieving her set objectives at work place. In the institutions of learning, a client may want to address the issue of low self-esteem that is affecting his relationship with others. Maybe the poor grades are as a result of poor time management. Hence proper time management would be the goal of therapy. Another example, a student goes for counselling because she fell pregnant during the COVID season, since there was no school. The goals of therapy would be to cope with the pregnancy, how to attend prenatal clinic without feeling stigmatized, how to raise her self-esteem, how to cope with academics in her state, and maybe how to prevent a repeat of premarital sex and pregnancy while at school. Therefore, the counsellor is at a position to help the client since there are clear focused SMART (Specific, Measurable, Achievable, Realistic and Time) goals

In crisis counselling the goal of therapy would Restoration of some sense of control Mastery of safety after crisis Restoration of coping capacities and subsequently preventing PTSD. Not always the client will be open to formulating goals of therapy. Resistance may be experienced when the very area that needs modification is bringing out desired consequences to the client. For example, a Form four girl would not want to leave her boyfriend (despite the fact that this relationship is ruining her studies) because apart from the fact that having a rich boyfriend is helping her win admiration from her fellow student, the boyfriend also provides some necessities like sanitary pads that the girl cannot get from her parents. At such a point the counsellor needs to use high challenge, High support skills.

Stage 4-Counselling intervention

After treatment plan and laying of the goals down the counsellor embarks on counselling intervention (Mometrix 2020) To begin with the counsellor makes a summary of the problem, that

is gets key points, of Identifies a strategy, and finally implementation. Summarization of the problems involves:

Step 1: Summary of the problem

Affective component: This is the domain of feelings. What feelings aroused by the issue? How does the issue affect the on the feelings? Where is the emotion located in the body? For example, a tight chest, loneliness in the head, emotional fatigue and the lower back. How would the client wish to feel? What feelings need to change. The five broad categories of feelings are anger fear sadness disgust enjoyment. Note also the thoughts because the thoughts trigger the feelings. For example, who thinks his boss being rude and unbearable may feel angry by the entire job.

Behavioral component: This involves the actions of the client as affected by the issue. How are the behaviors reinforcing the issue? Make a list of behavior that the client needs to change to reduce the effects of the problem

Cognitive Component What thoughts do you notice the client is having? How are the thoughts part of the problem? Are they irrational? What beliefs does the client hold? How can the thoughts be sieved to weed out the distortions? For example, in crisis counselling a client who has had a miscarriage, might have irrational thoughts such as, “I will never be able to have a child again”, I am not worthy a woman at all”, “Life is unfair” among others. The counsellor can use Cognitive

Behavior Therapy to challenge the irrational thoughts that are responsible for the negative emotional and behavioral patterns. (https://youtu.be/9c_Bv_FBE-c)

Interpersonal /Systemic Component: How does the client relate with others? The family relationship, work place relationships? Does he have friends? His peers at school? How is the

problem related to the interactions? How do the others view the problem? How does the client relate to the opposite sex? For example, at work place, how does he relate with others? How does he treat the juniors? Or maybe is he isolated? At times however clients who have family problems may mask at work place or in social gatherings. They hide their feelings such that everybody else apart from the spouse do not understand them. They put on a face that is peaceful and contented with life. In reality therefore the person would be retreating hiding isolating themselves or running away from the emotion. This in turn however causes not only psychological harm but also physical body may be affected. Research shows that these pent-up emotions has a direct relationship with High Blood Pressure, diabetes and general malaise.

Step 2-Identify the most helpful strategy for the client: This includes counselling approaches. (Kabir, S. M. (2017). For example, if cases relate to cognitive distortions, then the counsellor can use Cognitive Behavior therapy. If issues of childhood and unconscious material, psychoanalytic applies. Others are humanistic approaches, behavioral therapies or family therapy techniques can be used accordingly. The counsellor should explain to the client about the approach so that the client can be aware of what is going on. Also, client factors such as level of functioning should be considered. For example, the client mentioned above on PTSD characterized by loss of touch

with reality as a result of domestic violence and loss and grief. I had to refer her to a psychiatrist first for medication. Later on, I will see her for sessions. Since human beings are complex, so are their challenges it is not uncommon that clients come in for therapy with multiple issues. In that case, the most appropriate strategy to use is eclectic therapy (Serrao,2019). This is systematic combination of all therapeutic approaches. For example, a student who has discipline problem at

school may be diagnosed with antisocial personality disorder. On further exploration, there are family problems and still deeper exploration reveals low self-esteem. In such case therefore the counsellor has to use some of family therapy techniques. Cognitive Behavior therapy to challenge crooked thinking and Humanistic approaches to help the client build on self-esteem Behavioral therapy techniques of assignments and HomeWorks to modify behaviors are key in therapy.

Step 3: Implement the intervention strategy

So, the strategy should match the client case of the client. The goal of humanistic therapy is to help the client overcome feelings of inferiority and be able to express the emotions and feeling psychoanalysis focuses on bringing the unconscious materials to the conscious so as to deal with them. Cognitive behavior theory focuses on reducing distress and maladjusted behavior by correcting erroneous thoughts and beliefs, perceptions. Mindful self-compassion techniques aim at reducing anxiety and stress through embracing self and the difficult emotions. Gerald Egan's model seeks to establish the current scenario, preferred scenario and how to get there. **Kabir, Syed**

Muhammad. (2017). At this point when the client has fully explored his issues, understood where the problem is they can now act in the desired ways to achieve fully functional life.

Stage 5: Evaluation, Termination or referral

Termination ideally is done Either when the goals of therapy have been met. If the counsellor is not able to handle the client's case can refer appropriately or the problem has become more manageable/resolved (McLeod & McLeod 2011). Termination should be discussed at the beginning sessions. The counselor needs to inform the client how long the session takes. Remind the client like 2 to 3 sessions before termination. This helps them to disengage little by little. The following questions could be helpful to prepare the client for termination:

- Of what we've done so far, what has been the most meaningful or valuable to you?
- What have you learnt or accomplished?
- What positive change have you noticed in your life?
- How will you continue to use what you've learned?
- What do you want to remember from therapy?
- How do you think, will you look back on our work?
- What areas do you feel you may need to look into in future?

It is also important for the counsellor to evaluate self. Maybe the counsellor can also ask themselves if they have helped the client to be dependent. It is always fulfilling when the counsellor feels they have enabled the client to walk the journey of life victoriously.

THERAPY DOCUMENTATION

Therapy documentation is an indication that therapy took place, the documents are: Intake Forms, Case notes Therapy letters. Referral notes, Supervisors' reports and therapy reports. Documentation creates direction to follow. A well written report provides accountability for the counsellor and the referral agency. Sharing of the report depends on the circumstances for example at work place, the counsellor can share with the manager or the Human Resource manager. For

example, a worker may have been referred maybe because of habitual lateness at work, chronic absenteeism and therefore to be reinstated the therapist would need to share the report for proper handling of the case. Likewise, in Educational institutions therapy report may be shared to the principal, head of department, board of management or any other relevant authorities in charge of the student. For example, I have seen University students on different occasions regarding exam cheating. After the sessions I was to write the report and address it to the Deputy Vice Chancellor (DVC) academics. Other cases involving Secondary school students on discipline issues, I have written a report to the Principal.

Ethical principles Reporting/Recording /Documentation process

These ethical principles not only guide the recording process but also the entire counselling process.

Autonomy

Allow for self-determination. The client should be duly informed about the report. Before the report is handed to the relevant authority, the client should be aware of the content. Any issues arising should be discussed before handing in the report.

Justice

No bias/fairness

Fidelity

Keep promises as promised. For example, when you promise the boundaries of confidentiality, it should be kept. So, in case any document or information should be shared it should have been communicated with the client. The client should be made aware how this is beneficial to him and does not cause harm in case there is some controversy

Veracity

Truth fullness. Being honest. If the documents need to be shared the counsellor should be honest with the client. They should not lie to the client regarding the need for the documents' sharing. For example, if therapy report needed by the police in case of investigations, the client should not be told it is needed by the principal.

Confidentiality

No divulging of information Boundaries should be communicated clear. For example, in case of client self-harm or intends to harm somebody else, this should be disclosed.

Beneficence

This means that all actions should be for the benefit of the client. The client should be made to realize that though the report may not be pleasing, it is for his benefit.

Nonmaleficence

Whatever is done there should no harm should be intended against the client,

Note that the ethical principles work in harmony with each other. Key among them being Beneficence and Nonmaleficence. In fact, when faced with an ethical dilemma, the counsellor should always take the position that is for the benefit of the client and does not intend harm

ADMINISTRATION OF COUNSELLING SERVICES

This segment seeks to address how counselling services are managed in the institutions and organizations. This is not limited to school and work places. It is also extended to the overseeing counselling services in private agencies. This entails Overseeing of day to day operations of counselling services, Counselling services policies and implementation advocacy for Counselling services.

In the work place the administrator in charge is responsible for provision of supplies like stationery, soft tissues, filling system, water and all other amenities required for the services. The manager is also responsible for provision of equipment and facilities needed. For example, counselling rooms, offices, class/Training rooms. In learning institutions, the institution administration is responsible for the provision of amenities needed for guidance and counselling. The counsellors in charge should co-work with the management of the organizations and institutions to develop a check list of amenities and facilities required for smooth provision of the services.

More so counselling professionalism should be enhanced by the management by making sure that counselors get quality supervision (Ells, 2000). In Kenya most companies do not have a counsellor let alone a supervisor. However, this is very necessary. Counselling plays a key role in the mental health wellbeing of the employees hence translating into profits for the company. Likewise, in schools, there should be counsellors and counsellor supervisors who do not have other teaching roles in school to enable maximum concentration on the student and staff psychological wellbeing.

Since counselling is done with ethical guidelines, it is important to heed to the laid down policies by the relevant bodies mandated by the government to oversee counselling. For example in Kenya, the national body for counselling services regulation is Kenya Counsellors and Psychologist Association (KCPA). So, all counsellors should be registered with the organization. Other Organizations that regulate counselling services are Ministry of Education and Ministry of Health. So, organizations should seek to understand the procedures stipulated by these bodies

According to Osinbajo, Omotayo & Adeniyi, (2012) just like employees, Counsellors care and wellbeing should also be fostered by the institutions. One way is regular clinical supervision. Other ways are just like other employees, the counsellor is entitled to better pay according to his job group, better housing, medical cover, relevant, leaves and maybe insurance cover by the company. This will motivate the counsellor and hence good results of his work.

CONCLUSION

In this course I have covered the various counselling services offered by institutions and organizations and the process of counselling but contextualized it in work place, Educational institutions and crisis counselling. Towards the last, I have also talked about overseeing counselling services. For better delivery of services, there must be proper administration. Supplies, facilities and resources should be adequate. And policy guidelines should be adhered to. Moreover, counsellor's wellbeing should be taken care of for effective delivery of services. For clarity of points, I have given various examples of real clients and hypothetical clients. For the real clients, I have kept their personal details hidden for the ethical practice of confidentiality. Any institution focused on production and staying afloat must take into consideration of the mental wellbeing of the staff and the students. With increasing number of crisis incidents, organizations need to up their game by facilitating training counsellors on crisis counselling. It is true that Mental Health is wealth and that there is no health without mental health.

REFERENCES

- Adam O. Horvath (2005) *The therapeutic relationship: Research and theory*, *Psychotherapy Research*, 15:1-2, 3-7, DOI: [10.1080/10503300512331339143](https://doi.org/10.1080/10503300512331339143)
- Adeniji, A. A. and Osibanjo, Adewale Omotayo and Abiodun, A. J. (2013) Organisational Change and Human Resource Management Interventions:an Investigation of the Nigerian Banking Industry. *Serbian Journal of Management*, 8 (2). pp. 2-16.
- American Counselling Association (ACA) (n.d) Crisis Counselling
<https://www.counseling.org/docs/trauma-disaster/fact-sheet-10---1on1-crisis-counseling.pdf>
- American Counselling Association (December 1, 2012) The recipe for truly great counseling
<https://ct.counseling.org/2012/12/the-recipe-for-truly-great-counseling/>
- Bradley University (n.d) *Building trust with clients*
<https://onlinedegrees.bradley.edu/blog/building-trust-with-counseling-clients/>
accessed on 1/11/2020
- Center for Substance Abuse Treatment. *Clinical Supervision and Professional Development of the Substance Abuse Counselor*. Treatment Improvement Protocol (TIP) Series 52. DHHS Publication No. (SMA) 09-4435. Rockville, MD: Substance Abuse and Mental Health Services Administration, 2009.
- Ells, Tracy. (2000). *Handbook of Psychotherapy Supervision*. Psychotherapy Research - PSYCHOTHER RES. 10. 115- 117. [10.1093/ptr/10.1.115](https://doi.org/10.1093/ptr/10.1.115).
- Ethical Principles in Counselling <https://www.slideserve.com/paul/6-ethical-principles-in-counseling> Accessed on 30/10/2020
- Feltam (July 2015) **Counselling Psychology in the UK: a critical-humanistic perspective**
<https://www.tandfonline.com/doi/abs/10.1080/03060497.2015.1053691>
- Kabir, Syed Muhammad. (2017). COUNSELING APPROACHES.
[Researchgate.net/publication/325844296_COUNSELING_APPROACHES](https://www.researchgate.net/publication/325844296_COUNSELING_APPROACHES)

Kirti(2016) Need for psychological counselling for students Kirti(2016)

<https://www.slideshare.net/KirtiMatliwala/need-of-psychological-counseling-for>

McLeod J & McLeod J (2011). *Counselling Skills A practical guide for counsellors and helping professionals* (2nd ed). Maidenhead. Berkshire Open University Press McGraw-Hill Education

[Mometrix Test Preparation](#) (March 16, 2020)The 5 Main Stages of Client Assessment

<https://www.mometrix.com/academy/life-stages-in-client-assessment/>

[Mometrix Test Preparation](#)(March 16, 2020) Basic Counselling Skills

<https://www.mometrix.com/academy/basic-skills-of-a-counselor/>

Palmer S,(2 April 2015) *The Beginner's Guide to Counselling & Psychotherapy* . London. Sage Publications.

Relationship counselling *Dealing with Feelings*

<https://www.allrelationshipmatters.com.au/insights-healthy-relationships/why-talk-about-feelings> Accessed on 1/11/2020

Rolon-Arroyo, Benjamin & Oosterhoff, Benjamin & Layne, Christopher & Steinberg, Alan & Pynoos, Robert & Kaplow, Julie. (2019). The UCLA PTSD Reaction Index for DSM-5 Brief Form: A Screening Tool for Trauma-Exposed Youths. *Journal of the American Academy of Child & Adolescent Psychiatry*. 59. 10.1016/j.jaac.2019.06.022215.

Satterfield J, M. (2018) *How to Set SMART Goals with Cognitive Behavioral Therapy*

<https://www.youtube.com/watch?v=DzslQOcmuxM>

Serrao, Ferdie. (2019). Egan's Skilled Helper Model.

https://www.researchgate.net/publication/337084700_Egan's_Skilled_Helper_Model/citation/download

Sweeten J(2019)*Trauma Treatment Toolbox* : 165 Brain-Changing Tips, Tools & Handouts To Move Therapy Forward.Presi.inc <https://www.bookdepository.com/Trauma-Treatment-Toolbox-Jennifer-Sweeton/9781683731795>

Tyrell M (2019) *How to Help Your Clients Achieve Their Therapy Goals*

<https://www.youtube.com/watch?v=yRImwn85vD4>

Wango, Geoffrey. (2015). *Guidance and Counselling Services in Schools in Kenya*

https://www.researchgate.net/publication/322628940_Guidance_and_Counselling_Services_in_Schools_in_Kenya_Dr_Geoffrey_Wango

